

Behavioral health stakeholder map: case manager as coordination hub, showing workflow dependencies, authority gaps, and manual coordination points					
Stakeholder	Role in system	Key responsibilities	Workflows & dependencies	Pain points / where it breaks	Authority vs responsibility
Case manager Coordination hub High influence Highest burden	Bridges every care setting. Accountable for continuity but structurally dependent on cooperation from actors they cannot direct.	Post-discharge risk monitoring; care plan coordination; referral tracking; crisis triage; payer documentation; caregiver liaison; appointment follow-up	<i>Depends on:</i> inpatient discharge summary (often late/incomplete); outpatient session notes; caregiver reports; crisis team outcomes <i>Manual</i> Tracks patient contact, risk flags, and follow-up via phone, fax, and often personal spreadsheets when EHRs don't communicate <i>Workflow gap:</i> No automated alert when a patient misses a first post-discharge appointment — case manager must manually call to verify	Cannot prescribe, authorize services, or override payer decisions. Accountable for outcomes they cannot control. Caseloads of 40–80 patients mean risk stratification is often intuitive, not systematic. Critical information (crisis contact, ED visit, missed appointment) arrives days late or not at all.	Authority ■■■■■ Responsibility ■■■■■ ⚠ Severe authority gap. Held accountable for a system they cannot direct.
Inpatient team Acute stabilization High authority Short-term focus	Stabilizes and discharges. Operates in a silo — little visibility into patient's community context before or after admission.	Psychiatric evaluation; medication initiation; safety planning; discharge summary; warm handoff coordination (in theory)	<i>Sends to case manager:</i> discharge summary — often generated at discharge, not in advance <i>Manual</i> Warm handoffs to outpatient rely on phone/fax; no shared EHR in most systems <i>Dependency failure:</i> Bed-day pressure from payer concurrent review drives premature discharge before community follow-up is confirmed	Discharge summary arrives after the patient is already home — case manager is managing blind. Inpatient team rarely knows if the patient attended their first outpatient appointment. Prior community context (triggers, housing, support) often absent at admission.	Authority ■■■■■ Responsibility ■■■■■ ⚠ Authority fits acute role. Accountability ends at discharge — creating a handoff cliff.
Outpatient providers Ongoing treatment Med–high authority Fragmented data	Manages long-term stability. First 7–30 days post-discharge is the highest-risk window — and the hardest to bridge.	Medication management; therapy; relapse monitoring; crisis risk documentation; referrals to ancillary services	<i>Depends on:</i> inpatient summary (rarely received before first visit); case manager updates; patient self-report <i>Manual</i> Case manager calls to confirm patient attended appointment; provider faxes notes back <i>Systemic gap:</i> No-show at first post-discharge visit is a leading predictor of readmission — but no automated flag reaches the case manager	15–30 min slots insufficient for psychosocial risk assessment in high-acuity patients. Does not know about crisis contacts or ED visits unless case manager manually relays this. Documentation driven by billing requirements, not clinical communication.	Authority ■■■■■ Responsibility ■■■■■ ⚠ Mild gap. Responsible for continuity without reliable access to cross-setting data.
Crisis response team Acute intervention High urgency Often isolated	Intervenes at peak risk. Structurally disconnected from the patient's ongoing care — operates on incomplete history.	Mobile crisis assessment; de-escalation; safety planning; diversion from ED/inpatient; post-crisis handoff	<i>Activated by:</i> patient, caregiver, 988, outpatient provider — often without case manager awareness <i>Manual</i> Crisis outcome communicated to case manager by phone if at all; not auto-populated into HER <i>Dependency failure:</i> Case manager may learn about a crisis contact 48–72 hours later, or never, if patient doesn't self-report	Responds without access to care plan, current medications, or risk history. Post-crisis follow-up protocols vary widely — often no structured handback to case manager. Crisis contacts are rarely integrated into the patient's longitudinal record.	Authority ■■■■■ Responsibility ■■■■■ ⚠ Gap in information, not authority. Acts decisively but without the context needed to act safely.
Caregiver / family Informal surveillance High stake Structurally excluded	De facto early warning system. Observes daily risk but is excluded from formal care communication by consent and system design.	Daily monitoring; medication prompts; transport; crisis recognition; reporting changes to case manager	<i>Reports to:</i> case manager (phone/text, ad hoc); may contact crisis line or ED independently <i>Manual</i> No structured channel — caregiver calls case manager's direct line when concerned <i>Dependency failure:</i> Caregiver burnout or poor case manager availability means warning signs go unreported until a crisis	HIPAA/consent barriers mean caregivers often cannot receive clinical updates even when they are the patient's primary support. Only engaged reactively — no proactive check-ins, no formal role in the care plan. Burnout and compassion fatigue go unaddressed; the system treats them as a resource, not a stakeholder.	Authority ■■■■■ Responsibility ■■■■■ ⚠ Severe gap. Bears daily care burden with no formal role or support from the system.
Patient Care recipient Highest stake Least system power	Center of the system on paper. In practice, bears the coordination burden when the system fails — navigating multiple providers, payers, and crises with minimal support.	Attending appointments; self-reporting symptoms; medication adherence; communicating with multiple providers; managing insurance appeals	<i>Interacts with:</i> case manager (primary contact); outpatient provider; caregiver; crisis line; ED <i>Manual</i> In fragmented systems, the patient becomes the de facto information carrier between providers <i>Post-discharge window:</i> 7 days after inpatient discharge is the highest-risk period — follow-up appointment often not scheduled before discharge	Receives inconsistent information from multiple providers with no single source of truth. Insurance denials and prior auth delays interrupt medication or therapy at the worst moments. Cognitive and psychiatric symptoms make self-advocacy and care navigation hardest exactly when it matters most.	Authority ■■■■■ Responsibility ■■■■■ ⚠ Severe gap. Navigates a complex system with the fewest structural supports.
Insurer / payer Resource gatekeeper Highest authority Lowest accountability	Determines what care is financially possible. Exerts more control over care trajectories than any clinical actor — with no accountability for outcomes.	Prior authorization; concurrent inpatient review; utilization management; claims adjudication; care management programs (often parallel to, and uncoordinated with, clinical teams)	<i>Receives from:</i> case manager and outpatient team (documentation for auth/billing) <i>Controls:</i> inpatient length of stay; outpatient visit caps; medication formulary; referral approvals <i>Manual</i> Case manager spends significant time on prior auth calls and appeals — hours not spent on clinical coordination	Concurrent inpatient review drives discharge timing based on financial criteria, not clinical readiness. Prior auth delays for medications or outpatient services create gaps exactly at transitions. Payer care managers operate in parallel to clinical case managers — often duplicating outreach without sharing information.	Authority ■■■■■ Responsibility ■■■■■ ⚠ Inverse gap. Maximum control over care with near-zero accountability for outcomes.

Manual Manual coordination required — a system breakdown point
 ■ Responsibility (accountability for outcomes, 1–5)

■ Authority (formal decision power, 1–5)

⚠ = Authority Gap